

PEARSON EDEXCEL GCSE (9–1)

HISTORY • PAPER 1: THEMATIC STUDY & HISTORIC ENVIRONMENT Time: 1 hour 15 minutes (Full Mock Simulation / Guided Practice) • Total Marks: 52 MEDICINE IN BRITAIN, C1250–PRESENT AND THE BRITISH SECTOR OF THE WESTERN FRONT, 1914–18 Complete 32-Page Mastery Exam Compendium Option 11 • Section B (Thematic Study, c1250–present) & Section A (Historic Environment, 1914–18)	PAPER REFERENCE 1H10/11
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COMPLETE 32-PAGE SPECIFICATION PRACTICE TRACKER & HOMEWORK AUDIT			Track marks achieved across Section B (c1250–present) & Section A (Western Front)			
HW SET / DUE		QUESTION & FORMAT	SPECIFICATION FOCUS & HISTORICAL CONTENT	PAGE	MARKS	SCORE
SECTION B: MEDIEVAL MEDICINE (C1250–C1500)						
Set: _____ Due: _____	Q3 · Similarity	Ideas about cause of illness (Medieval vs Renaissance continuity)	2	4	[] / 4	
Set: _____ Due: _____	Q4 · Explain Why	Continuity in medical treatments in Medieval England (Church & Galen)	2	12	[] / 12	
Set: _____ Due: _____	Q5/6 · Statement Essay	Theory of Four Humours as dominant medical foundation (c1250–c1500)	3	16+4	[] / 20	
SECTION B: RENAISSANCE MEDICINE (C1500–C1700)						
Set: _____ Due: _____	Q3 · Difference	Responses to Black Death (1348) vs Great Plague (1665)	4	4	[] / 4	
Set: _____ Due: _____	Q4 · Explain Why	Rapid change in anatomical understanding (Vesalius & Printing Press)	4	12	[] / 12	
Set: _____ Due: _____	Q5/6 · Statement Essay	Thomas Sydenham's observation vs Royal Society vs Vesalius/Harvey	5	16+4	[] / 20	
SECTION B: 18TH & 19TH CENTURY SURGERY (C1700–C1900)						
Set: _____ Due: _____	Q3 · Difference	Surgery in 18th C vs late 19th C antiseptic operating theatres	6	4	[] / 4	
Set: _____ Due: _____	Q4 · Explain Why	Rapid transformation in surgery 1840–1900 (Simpson, Lister, Asepsis)	6	12	[] / 12	
Set: _____ Due: _____	Q5/6 · Statement Essay	Pasteur's Germ Theory vs 1875 Public Health Act vs Antibiotics	7	16+4	[] / 20	
SECTION B: 18TH & 19TH CENTURY PUBLIC HEALTH & CHOLERA						
Set: _____ Due: _____	Q3 · Similarity	Public health legislation: 1848 Public Health Act vs 1875 Act	8	4	[] / 4	
Set: _____ Due: _____	Q4 · Explain Why	Government involvement in public health 1850–1900 (Chadwick, Snow, 1875 Act)	8	12	[] / 12	
Set: _____ Due: _____	Q5/6 · Statement Essay	John Snow's Broad Street cholera work vs 1875 Act vs Bazalgette's Sewers	9	16+4	[] / 20	
SECTION B: MODERN BRITAIN: ANTIBIOTICS & THE NHS (C1900–PRESENT)						
Set: _____ Due: _____	Q3 · Difference	Early 20th C magic bullets (Salvarsan 606) vs modern antibiotics	10	4	[] / 4	
Set: _____ Due: _____	Q4 · Explain Why	Mass production of penicillin 1938–45 (Florey, Chain, US Govt)	10	12	[] / 12	
Set: _____ Due: _____	Q5/6 · Statement Essay	Aneurin Bevan & The NHS (1948) vs DNA Structure vs High-Tech Care	11	16+4	[] / 20	
SECTION B: 21ST CENTURY SCIENCE, GENETICS & PREVENTION						
Set: _____ Due: _____	Q3 · Difference	19th C diagnostic methods vs 20th C imaging technology (CT/MRI)	12	4	[] / 4	
Set: _____ Due: _____	Q4 · Explain Why	Rapid developments in disease prevention after 1900 (Vaccines & Lifestyle)	12	12	[] / 12	
Set: _____ Due: _____	Q5/6 · Statement Essay	DNA discovery (1953) vs Mass Penicillin vs Foundation of the NHS	13	16+4	[] / 20	
SECTION B: SYNOPTIC & CROSS-PERIOD THEMATIC MASTERY						
Set: _____ Due: _____	Q3 · Similarity	Government intervention: 1348 Black Death vs 1848 Cholera	14	4	[] / 4	
Set: _____ Due: _____	Q4 · Explain Why	Role of Science & Technology across medicine c1250–present	14	12	[] / 12	
Set: _____ Due: _____	Q5/6 · Statement Essay	Individual genius vs Government action in medical progress c1500–present	15	16+4	[] / 20	
Set: _____ Due: _____	Q3 Mastery Lab	The 4-Mark Comparison Lab: 3 Cross-Period Pairing Drills	16	12	[] / 12	
Set: _____ Due: _____	Q5/6 Paired Choice	Section B Timed Simulation: Paired Choice Strategy & Live Mock	17	16+4	[] / 20	
SECTION A: THE WESTERN FRONT, 1914–18 (HISTORIC ENVIRONMENT)						
Set: _____ Due: _____	Set 1 · Features & Q2b	Somme: Features of CCS/Field Amb. & Transport Follow-up Grid	18	8	[] / 8	
Set: _____ Due: _____	Set 1 · Q2a Utility	Somme: Sources A & B Utility (Rev. Davies vs Vera Brittain VAD)	19–20	8	[] / 8	
Set: _____ Due: _____	Set 2 · Features & Q2b	Ypres: Features of Gas/Gangrene & Passchendaele Stretcher Relays	21	8	[] / 8	
Set: _____ Due: _____	Set 2 · Q2a Utility	Ypres: Sources C & D Utility (8th Field Amb. vs Sister Luard QAIMNS)	22–23	8	[] / 8	
Set: _____ Due: _____	Set 3 · Features & Q2b	Arras: Thompson Cave Hospital, Carrel-Dakin & Tunnel Record Grid	24	8	[] / 8	
Set: _____ Due: _____	Set 3 · Q2a Utility	Arras: Sources E & F Utility (Major Myers RAMC vs IWM Tunnel Photo)	25–26	8	[] / 8	
Set: _____ Due: _____	Set 4 · Features & Q2b	Cambrai: Blood Transfusion, Thomas Splint & Medical Diary Follow-up	27	8	[] / 8	
Set: _____ Due: _____	Set 4 · Q2a Utility	Cambrai: Sources G & H Utility (Capt. Robertson Blood vs Orderly Splint)	28–29	8	[] / 8	
REFERENCE & OFFICIAL EXAMINER PROTOCOLS						
Independent Revision	Masterclass & Matrix	Primary Source Provenance Typology (p. 30) & Examiner Pitfalls (p. 31)	30–31	—	MASTERED	
Specification Mastery	Specification Audit	Word-for-word syllabus checklist covering Option 11 in its entirety	32	—	AUDITED	

3

EDEXCEL JUNE 2019

Explain one way in which ideas about the cause of illness in the Medieval period (c1250–c1500) were similar to ideas about the cause of illness in the Renaissance (c1500–c1700).

[4]

KEY KNOWLEDGE (AO1) Four HumoursMiasma (Bad Air)Galen / Hippocrates1348 Black Death1665 Great Plague	CAUSAL CONNECTIVES • One clear similarity was... • In the medieval period, physicians believed... • Similarly, in the Renaissance...	MODEL SENTENCE / ANSWER GUIDE (AO1/AO2) "One similarity was that both eras believed miasma caused disease; in 1348 people carried sweet herbs to purify bad air, and in 1665 doctors still wore beak masks stuffed with aromatic herbs to ward off the plague."
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4

UNEXAMINED SPEC TARGET

Explain why there was so little change in medical treatments in Medieval England between c1250 and c1500.

[12]

You may use the following in your answer:

• The Christian Church • The continuing influence of Galen and Hippocrates

(You must also use information of your own.)

FACTOR BANK (AO1) Church DogmaGalen MonotheismDissection BansRoger Bacon JailedBleeding & Purging	CAUSAL CONNECTIVES • A primary obstacle to progress was... • The Church enforced this by... • Consequently, treatments remained...	3 P-E-E PARAGRAPH PLAN Para 1: The Catholic Church → Para 2: Galen's Unchallenged Authority → Para 3: Lack of Scientific Tech.
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PARAGRAPH 1: THE INFLUENCE OF THE CHRISTIAN CHURCH (STIMULUS 1)

PARAGRAPH 2: GALEN'S ENDURING AUTHORITY & HUMERAL ORTHODOXY (STIMULUS 2)

PARAGRAPH 3: OWN KNOWLEDGE — SOCIAL HIERARCHY & LACK OF SCIENTIFIC INSTRUMENTS

5 / 6 ★ HIGH-YIELD FORECAST 'Belief in the Theory of the Four Humours was the most important reason why people failed to treat the Black Death successfully in 1348–49.' How far do you agree? Explain your answer. **[16+4]**

You may use the following in your answer: • **Bloodletting and purging (Opposites)** • **Religious explanations and flagellants** (You must also use information of your own.)

ARGUMENT BANK (AO1/AO2) Side A: Four Humours Phlebotomy, purging, hot/cold herbs; weakened patients. Side B: Religion/Miasma God's punishment, flagellation, bad air, sweet herbs. Side C: Filth & Rats Fleas on black rats (Yersinia pestis); total ignorance of microbes.	EVALUATIVE CONNECTIVES • <i>On the one hand, humeral theory...</i> • <i>However, religious fatalism was more damaging...</i> • <i>Crucially, the decisive factor was...</i>	CRITERIA-LED JUDGEMENT Weigh the scale of influence : Did humeral theory fail patients more, or did religious fatalism prevent municipal sanitation?
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INTRODUCTION & PARAGRAPH 1: AGREE WITH NAMED FACTOR (FOUR HUMOURS BLEEDING & PURGING)

PARAGRAPH 2: COUNTER-FACTOR — RELIGIOUS EXPLANATIONS, GOD'S WRATH & FLAGELLANTS

PARAGRAPH 3: ALTERNATIVE FACTOR — MIASMA, BAD AIR & COMPLETE LACK OF GERM KNOWLEDGE

CONCLUSION: SUPPORTED FINAL JUDGEMENT (CRITERIA: WHICH FACTOR WAS MOST DECISIVE?)

3

EDEXCEL JUNE 2018

Explain one way in which the response of authorities to the Great Plague (1665) was different from the response of authorities to the Black Death (1348).

[4]

KEY KNOWLEDGE (AO1) 1348 Black Death No quarantine; church-led prayer; King street order. 1665 Great Plague Mayor quarantine; red crosses & watchmen; searchers; mass pits.	CONTRAST CONNECTIVES - One clear difference in response was... - In 1348, authorities were largely powerless... - In contrast, by 1665 the Lord Mayor enforced...	MODEL SENTENCE / ANSWER GUIDE (AO1/AO2) "In contrast to 1348 when authorities took little organized action, by 1665 the Mayor of London enforced systematic quarantine, locking infected families in their homes with red crosses painted on doors and watchmen outside."
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4

EDEXCEL JUNE 2022

Explain why there was rapid change in anatomical understanding in the period c1500–c1700.

[12]

You may use the following in your answer: • Andreas Vesalius and The Fabric of the Human Body (1543) • The invention of the movable-type printing press (c1450s) (You must also use information of your own.)		
FACTOR BANK (AO1) Vesalius (300+ Galen Errors) Human Dissection Movable Type Print William Harvey (1628 Heart) Mechanical Pumps	CAUSAL CONNECTIVES - A decisive breakthrough came when... - By directly dissecting human corpses, Vesalius... - The printing press multiplied this impact by...	3 P-E-E PARAGRAPH PLAN Para 1: Vesalius & Direct Dissection → Para 2: The Printing Press & Spread of Texts → Para 3: Harvey & Circulation.

PARAGRAPH 1: ANDREAS VESALIUS & EMPIRICAL HUMAN DISSECTION (STIMULUS 1)

PARAGRAPH 2: THE PRINTING PRESS & SCIENTIFIC STANDARDIZATION (STIMULUS 2)

PARAGRAPH 3: OWN KNOWLEDGE — WILLIAM HARVEY & MECHANICAL HEART CIRCULATION (1628)

5/6SAMPLE ASSESSMENT MATERIAL

'Thomas Sydenham's work was the most significant turning point in medical care between c1500 and c1700.' How far do you agree? Explain your answer.

[16+4]

You may use the following in your answer:

• Thomas Sydenham and bedside clinical observation (Observationes Medicae, 1676)

• The Royal Society and its scientific motto 'Nullius in verba' (1660)

(You must also use information of your own.)

ARGUMENT BANK (AO1/AO2) Side A: Sydenham Grouped diseases by symptoms; rejected individual humours; cinchona bark for malaria. Side B: Royal Society Peer review; scientific journal (Philosophical Transactions); empirical method. Side C: Anatomy Titans Vesalius (1543) and Harvey (1628); dismantled Galenic dogma.	EVALUATIVE CONNECTIVES • Sydenham was revolutionary because... • However, the Royal Society provided institutional scale... • Yet neither transformed ordinary patient survival because...	CRITERIA-LED JUDGEMENT Distinguish between clinical observation at the bedside (Sydenham) vs theoretical anatomy (Harvey/Vesalius) vs treatments (persisting humours).
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INTRODUCTION & PARAGRAPH 1: AGREE WITH NAMED FACTOR (THOMAS SYDENHAM & OBSERVATION)

PARAGRAPH 2: COUNTER-FACTOR — THE ROYAL SOCIETY & INSTITUTIONAL EMPIRICAL SCIENCE

PARAGRAPH 3: ALTERNATIVE FACTOR — VESALIUS, HARVEY & THE OVERTHROW OF GALEN

CONCLUSION: SUPPORTED FINAL JUDGEMENT (CRITERIA: THEORY VS EVERYDAY PRACTICE)

3★ HIGH-YIELD FORECAST

Explain one way in which surgical procedures in the 18th century were different from surgical procedures in the late 19th century.

[4]

KEY KNOWLEDGE (AO1) 18th C Surgery Speed-focused (Liston 28s); conscious; apron pus; cautery iron. Late 19th C Surgery Anaesthesia (chloroform); antiseptic carbolic spray (Lister); aseptic gowns/gloves.	CONTRAST CONNECTIVES • A fundamental difference lay in... • In the 18th century, speed was paramount because... • In contrast, by the 1890s surgeons operated...	MODEL SENTENCE / ANSWER GUIDE (AO1/AO2) <i>"One major difference was the control of infection: in the 18th century, surgeons operated in blood-stained frock coats with unsterilized tools, whereas by the late 19th century, Joseph Lister's carbolic acid spray and aseptic autoclaves eliminated bacteria."</i>
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4EDEXCEL JUNE 2023

Explain why there was rapid change in surgical treatments in Britain between 1840 and 1900.

[12]

You may use the following in your answer: • James Simpson and chloroform (1847) • Joseph Lister and carbolic acid (1867) (You must also use information of your own.)

FACTOR BANK (AO1) James Simpson (1847) Queen Victoria (1853) Joseph Lister (Carbolic) Aseptic Surgery (1890s) Steam Autoclaves Catgut Ligatures	CAUSAL CONNECTIVES • The breakthrough of anaesthesia allowed... • However, longer operations triggered the 'Black Period' until... • Ultimately, aseptic methods ensured...	3 P-E-E PARAGRAPH PLAN Para 1: Simpson & Chloroform (Pain) → Para 2: Lister & Carbolic Acid (Infection) → Para 3: Aseptic Surgery & Ligatures.
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PARAGRAPH 1: JAMES SIMPSON & THE CONQUEST OF PAIN (ANAESTHESIA) (STIMULUS 1)

PARAGRAPH 2: JOSEPH LISTER & ANTISEPTIC SURGERY (DEFEATING INFECTION) (STIMULUS 2)

PARAGRAPH 3: OWN KNOWLEDGE — ASEPTIC SURGERY (STEAM AUTOCLAVES, RUBBER GLOVES & GOWNS)

5 / 6 EDEXCEL NOVEMBER 2020 'Pasteur's publication of the Germ Theory in 1861 was the most important turning point in the treatment of illness in the period c1700–present.' How far do you agree? Explain your answer. [16+4]

You may use the following in your answer: • The Public Health Act of 1875 • The development of penicillin (1928–1940s) (You must also use information of your own.)

ARGUMENT BANK (AO1/AO2) Side A: Germ Theory Ended spontaneous generation; Koch identified microbes; antiseptic surgery. Side B: 1875 Public Health Compulsory clean water, sewers, street cleaning; mass preventative impact. Side C: Penicillin Fleming (1928), Florey & Chain (1941); first mass cure for internal bacterial infection.	EVALUATIVE CONNECTIVES • While Germ Theory provided the scientific cause... • Nevertheless, it did not immediately cure patients until... • Therefore, the practical turning point was...	CRITERIA-LED JUDGEMENT Distinguish between theoretical understanding (identifying microbes) vs practical treatment (curing patients with antibiotics).
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INTRODUCTION & PARAGRAPH 1: AGREE WITH NAMED FACTOR (PASTEUR'S GERM THEORY & KOCH)

PARAGRAPH 2: COUNTER-FACTOR — THE 1875 PUBLIC HEALTH ACT & MUNICIPAL SANITATION (STIMULUS 1)

PARAGRAPH 3: ALTERNATIVE FACTOR — 20TH CENTURY ANTIBIOTICS (FLEMING, FLOREY & CHAIN) (STIMULUS 2)

CONCLUSION: SUPPORTED FINAL JUDGEMENT (CRITERIA: LAB SCIENCE VS MASS PUBLIC SURVIVAL)

3

★ HIGH-YIELD FORECAST

Explain one way in which the Public Health Act of 1848 was similar to the Public Health Act of 1875.

[4]

KEY KNOWLEDGE (AO1) 1848 Act General Board of Health; town councils permitted to set up local boards; clean water. 1875 Act Compulsory clean water, sewers, medical officers, street lighting, food inspectors.	CAUSAL CONNECTIVES • One clear similarity was... • Both acts aimed to tackle urban filth by... • Similarly, each statute empowered councils to...	MODEL SENTENCE / ANSWER GUIDE (AO1/AO2) <i>"One clear similarity was that both statutes recognized the duty of government to improve sanitation; both the 1848 and 1875 Acts provided local authorities with the legal powers to supply clean water and manage sewer systems to reduce epidemic disease."</i>
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4

EDEXCEL JUNE 2021

Explain why there was a major change in government action on public health in the second half of the nineteenth century (1850–1900).

[12]

You may use the following in your answer: • John Snow and the Broad Street pump (1854) • The Public Health Act of 1875 <i>(You must also use information of your own.)</i>		
FACTOR BANK (AO1) John Snow (1854 Cholera) Great Stink (1858) Bazalgette Sewers 1867 Reform Act Pasteur Germ Theory 1861 Compulsory 1875 Act	CAUSAL CONNECTIVES • Scientific proof defeated miasma when... • Crucially, the extension of the vote in 1867 meant... • Consequently, laissez-faire was abandoned for...	3 P-E-E PARAGRAPH PLAN Para 1: John Snow & Water Contamination → Para 2: Compulsory 1875 Legislation → Para 3: 1867 Enfranchisement & Bazalgette.

PARAGRAPH 1: JOHN SNOW & THE DISPROOF OF MIASMA (STIMULUS 1)

PARAGRAPH 2: THE PUBLIC HEALTH ACT OF 1875 & THE END OF LAISSEZ-FAIRE (STIMULUS 2)

PARAGRAPH 3: OWN KNOWLEDGE — WORKING-CLASS ENFRANCHISEMENT (1867) & BAZALGETTE'S SEWERS

5/6★ HIGH-YIELD FORECAST

'John Snow's investigation of cholera in 1854 was the main reason why public health improved in nineteenth-century Britain.' How far do you agree? Explain your answer.

[16+4]

You may use the following in your answer: • The Broad Street pump investigation (1854) • Joseph Bazalgette and the London sewer network (1858–1875) (You must also use information of your own.)

ARGUMENT BANK (AO1/AO2) Side A: John Snow Spot map; removed pump handle; proved waterborne contagion before Germ Theory. Side B: Bazalgette 1,100 miles of street sewers; 82 miles of intercepting sewers; Great Stink 1858. Side C: State Action Edwin Chadwick's 1842 report; 1875 Act making clean water and sewer checks mandatory.	EVALUATIVE CONNECTIVES • Snow provided the decisive medical proof... • However, evidence was useless without massive civil engineering... • Ultimately, statutory government coercion was decisive because...	CRITERIA-LED JUDGEMENT Weigh epidemiological evidence (Snow) vs physical infrastructure (Bazalgette) vs national enforcement (1875 Act).
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INTRODUCTION & PARAGRAPH 1: AGREE WITH NAMED FACTOR (JOHN SNOW'S BROAD STREET INVESTIGATION)

PARAGRAPH 2: COUNTER-FACTOR — JOSEPH BAZALGETTE & CIVIL ENGINEERING (LONDON SEWERS)

PARAGRAPH 3: ALTERNATIVE FACTOR — CHADWICK'S 1842 REPORT & THE COMPULSORY 1875 ACT

CONCLUSION: SUPPORTED FINAL JUDGEMENT (CRITERIA: SCIENTIFIC PROOF VS STATUTORY ENFORCEMENT)

3 UNEXAMINED SPEC TARGET

Explain one way in which early twentieth-century 'magic bullets' were different from modern antibiotics.

[4]

KEY KNOWLEDGE (AO1) Magic Bullets Synthetic chemical dyes (Ehrlich's Salvarsan 606 in 1909 for syphilis; Domagk's Prontosil in 1932); targeted specific microbes but toxic. Antibiotics Naturally occurring moulds/substances (Fleming 1928, Florey & Chain 1940); destroyed bacterial cell walls safely.	CONTRAST CONNECTIVES • One fundamental difference in treatment was... • Early magic bullets were synthetic chemicals that... • In sharp contrast, antibiotics were derived from living...	MODEL SENTENCE / ANSWER GUIDE (AO1/AO2) <i>"Early magic bullets like Ehrlich's Salvarsan 606 (1909) were synthetic chemical dyes that attacked specific microbes like syphilis but were toxic in high doses, whereas modern antibiotics like penicillin were derived from living fungal mould and destroyed bacterial cell walls safely without poisoning healthy human tissue."</i>
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4 EDEXCEL JUNE 2021

Explain why there was rapid progress in the development and mass production of penicillin in the years c1938–c1945.

[12]

You may use the following in your answer: • Howard Florey and Ernst Chain at Oxford University • US Government funding and the Second World War (You must also use information of your own.)		
FACTOR BANK (AO1) Florey & Chain (1938–41) Mouse Trials (1940) Albert Alexander (1941) US War Production Board Corn Steep Liquor Deep Fermentation Tanks (1944)	CAUSAL CONNECTIVES • The scientific catalyst was Florey and Chain's purification... • However, British pharmaceutical firms lacked wartime capacity... • Consequently, US wartime intervention unlocked mass production...	3 P-E-E PARAGRAPH PLAN Para 1: Florey & Chain's Purification → Para 2: US State Intervention & Wartime Urgency → Para 3: Industrial Fermentation.

PARAGRAPH 1: HOWARD FLOREY, ERNST CHAIN & BIOCHEMICAL PURIFICATION (STIMULUS 1)

PARAGRAPH 2: US GOVERNMENT FUNDING & WARTIME MILITARY URGENCY (STIMULUS 2)

PARAGRAPH 3: OWN KNOWLEDGE — INDUSTRIAL INNOVATIONS (CORN STEEP LIQUOR, CANTALOUPE STRAIN & DEEP FERMENTATION)

5 / 6 ★ HIGH-YIELD FORECAST 'The creation of the National Health Service in 1948 was the most significant breakthrough in medical care between c1900 and the present.' How far do you agree? Explain your answer. **[16+4]**

You may use the following in your answer: • Aneurin Bevan and the founding of the NHS (5 July 1948) • The discovery of the structure of DNA by Watson, Crick and Franklin (1953) (You must also use information of your own.)

ARGUMENT BANK (AO1/AO2) Side A: The NHS (1948) Free at point of delivery; universal access; nationalized hospitals; ended two-tier care for working class. Side B: DNA & Genetics 1953 double helix; Human Genome Project; gene therapy and targeted oncological drugs. Side C: High-Tech Medicine Advanced surgical techniques (hip replacements, organ transplants, keyhole surgery, dialysis).	EVALUATIVE CONNECTIVES - The NHS revolutionized healthcare delivery by removing economic barriers... - However, social access was meaningless without clinical cures... - Ultimately, scientific breakthroughs like DNA were decisive because...	CRITERIA-LED JUDGEMENT Weigh democratic social access (NHS ensuring everyone could consult a doctor) vs scientific capability (DNA and biotech discovering cures).
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INTRODUCTION & PARAGRAPH 1: AGREE WITH NAMED FACTOR (ANEURIN BEVAN & THE FOUNDATION OF THE NHS)

PARAGRAPH 2: COUNTER-FACTOR — WATSON, CRICK & FRANKLIN'S DISCOVERY OF DNA STRUCTURE (STIMULUS 2)

PARAGRAPH 3: ALTERNATIVE FACTOR — HIGH-TECH SURGICAL & PHARMACEUTICAL BREAKTHROUGHS (ORGAN TRANSPLANTS & CHEMOTHERAPY)

CONCLUSION: SUPPORTED FINAL JUDGEMENT (CRITERIA: DEMOCRATIC SOCIAL ACCESS VS CLINICAL CURE)

3

UNEXAMINED SPEC TARGET

Explain one way in which methods of diagnosing illness in the nineteenth century were different from methods of diagnosing illness in the modern era (c1900–present).

[4]

KEY KNOWLEDGE (AO1) 19th C Diagnosis Relied on external physical symptoms, stethoscopes (Laennec), taking pulse, observing urine. Modern Diagnosis High-tech medical physics: X-rays, CT scans, MRI, endoscopy, blood tests, and genetic DNA profiling.	CONTRAST CONNECTIVES • One fundamental difference in diagnosis was... • In the 19th century, doctors were restricted to external... • In sharp contrast, modern clinical practice utilizes internal...	MODEL SENTENCE / ANSWER GUIDE (AO1/AO2) <i>"In the 19th century diagnosis was restricted to external physical examination using stethoscopes and feeling the pulse, whereas modern doctors use advanced medical physics such as CT and MRI scans alongside genetic blood tests to diagnose disease internally with microscopic precision."</i>
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★ HIGH-YIELD FORECAST

Explain why understanding of the causes of disease advanced rapidly after 1900.

[12]

You may use the following in your answer: • The discovery of the structure of DNA (1953) • Research into lifestyle factors (e.g. Doll and Hill on smoking and lung cancer, 1950) (You must also use information of your own.)		
FACTOR BANK (AO1) Watson, Crick & Franklin 1953 Human Genome Project (1990–2003) Doll & Hill Smoking Link 1950 Lifestyle Epidemiology Electron Microscopes (1931)	CAUSAL CONNECTIVES • A revolutionary advance was the discovery of genetic coding... • By uncovering DNA structure, scientists proved that disease... • Furthermore, statistical epidemiology demonstrated that lifestyle...	3 P-E-E PARAGRAPH PLAN Para 1: DNA Structure & Hereditary Diseases → Para 2: Lifestyle Research (Doll & Hill / Smoking) → Para 3: High-Tech Microscopy & Virology.

PARAGRAPH 1: GENETICS & THE DISCOVERY OF THE STRUCTURE OF DNA (STIMULUS 1)

PARAGRAPH 2: STATISTICAL EPIDEMIOLOGY & LIFESTYLE CAUSATION (DOLL & HILL / SMOKING) (STIMULUS 2)

PARAGRAPH 3: OWN KNOWLEDGE — SCIENTIFIC INSTRUMENTATION (ELECTRON MICROSCOPES & VIROLOGY)

5 / 6★ HIGH-YIELD FORECAST

'The discovery of the structure of DNA in 1953 was the most significant turning point in modern medicine (c1900–present).' How far do you agree? Explain your answer.

[16+4]

You may use the following in your answer: • The Human Genome Project (completed 2003) • The development of mass-produced penicillin in the Second World War (You must also use information of your own.)

ARGUMENT BANK (AO1/AO2) Side A: DNA & Genetics Watson, Crick, Franklin; mapping human genetic blueprint; BRCA breast cancer gene screening; targeted chemotherapy. Side B: Antibiotics Penicillin (Florey & Chain); defeated lethal bacterial infections (pneumonia, sepsis); saved tens of millions. Side C: State Prevention Mass immunization (polio, MMR); Clean Air Acts (1956); anti-smoking laws (2007 public smoking ban).	EVALUATIVE CONNECTIVES • <i>DNA provided the foundational scientific breakthrough of the century...</i> • <i>However, penicillin had a far more immediate impact on mass survival...</i> • <i>Consequently, while DNA holds future promise, antibiotics transformed...</i>	CRITERIA-LED JUDGEMENT Weigh immediate mortality reduction (penicillin curing acute infections) vs long-term structural understanding (DNA explaining incurable chronic conditions).
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INTRODUCTION & PARAGRAPH 1: AGREE WITH NAMED FACTOR (DNA DISCOVERY & HUMAN GENOME PROJECT)

PARAGRAPH 2: COUNTER-FACTOR — MASS-PRODUCED ANTIBIOTICS & ERADICATION OF BACTERIAL KILLERS (STIMULUS 2)

PARAGRAPH 3: ALTERNATIVE FACTOR — GOVERNMENT PUBLIC HEALTH & PREVENTATIVE LEGISLATION (CLEAN AIR & SMOKING BANS)

CONCLUSION: SUPPORTED FINAL JUDGEMENT (CRITERIA: IMMEDIATE ACUTE SURVIVAL VS LONG-TERM GENETIC MASTERY)

3 EDEXCEL NOVEMBER 2020

Explain one way in which the training of physicians in the Medieval period was similar to the training of physicians in the Renaissance period.

[4]

KEY KNOWLEDGE (AO1) Medieval Training Oxford & Cambridge; book-learning from Galen & Hippocrates; zero clinical dissection. Renaissance Training Continued reading of classical texts; Royal College of Physicians licensing; slow adoption of Harvey.	CONTINUITY CONNECTIVES • One clear continuity in physician training was... • In medieval universities, medical education was based on... • Similarly, in the Renaissance, mainstream physicians still...	MODEL SENTENCE / ANSWER GUIDE (AO1/AO2) <i>"In both eras, physician education was heavily theoretical based on classical books; medieval universities taught Galen's texts without dissection, and seventeenth-century medical schools still required doctors to master Galenic lectures before granting licensing."</i>
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4 ★ HIGH-YIELD FORECAST

Explain why the British Government abandoned its laissez-faire attitude towards public health in the nineteenth century.

[12]

You may use the following in your answer: • Edwin Chadwick's Report on the Sanitary Condition of the Labouring Population (1842) • The Second Reform Act giving working-class men the vote (1867) (You must also use information of your own.)		
FACTOR BANK (AO1) Chadwick Report 1842 Broad Street Cholera 1854 John Snow Water Pump 1867 Reform Act (Working Votes) 1875 Public Health Act	CAUSAL CONNECTIVES • A primary catalyst was empirical statistical evidence... • Chadwick proved that filthy living conditions caused economic loss... • Politicians were forced to act when working-class men gained the vote...	3 P-E-E PARAGRAPH PLAN Para 1: Chadwick's Report (Economic & Health Evidence) → Para 2: Political Enfranchisement (1867 Reform Act) → Para 3: Snow & Cholera.

PARAGRAPH 1: EDWIN CHADWICK & THE ECONOMIC BURDEN OF FILTH (STIMULUS 1)

PARAGRAPH 2: THE 1867 REFORM ACT & WORKING-CLASS POLITICAL PRESSURE (STIMULUS 2)

PARAGRAPH 3: OWN KNOWLEDGE — JOHN SNOW, BROAD STREET (1854) & COMPULSORY 1875 LEGISLATION

5 / 6 UNEXAMINED SPEC TARGET 'Individual genius was more important than government action in improving medicine and public health between c1250 and the present.' How far do you agree? Explain your answer. [16+4]

You may use the following in your answer: • William Harvey and the circulation of the blood (1628) • The Public Health Act of 1875 (You must also use information of your own.)

ARGUMENT BANK (AO1/AO2) Side A: Individuals Jenner, Simpson, Lister, Pasteur, Koch, Fleming; breakthroughs in clinical laboratory science. Side B: Government Action 1875 Public Health Act, Clean Air Acts (1956), NHS (1948); statutory power & funding. Side C: Synergy Individuals discover cures, but government must enforce and fund mass delivery.	EVALUATIVE CONNECTIVES - Individuals were indispensable for initiating progress... - However, individual ideas were impotent without state statutory power... - The decisive factor across 750 years was the institutional transition...	CRITERIA-LED JUDGEMENT Synthesize: Without individual science (Pasteur/Fleming), government had no solutions; but without government funding (US WWII/NHS), individuals could not mass-treat.
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INTRODUCTION & PARAGRAPH 1: AGREE WITH NAMED FACTOR (INDIVIDUAL GENIUS — HARVEY, JENNER, SIMPSON)

PARAGRAPH 2: COUNTER-FACTOR — GOVERNMENT ACTION, PUBLIC HEALTH ACTS & THE WELFARE STATE (STIMULUS 2)

PARAGRAPH 3: ALTERNATIVE FACTOR — WAR, INDUSTRIAL TECHNOLOGY & MASS COMMUNICATIONS

CONCLUSION: SUPPORTED FINAL JUDGEMENT (CRITERIA: SCIENTIFIC DISCOVERY VS STATUTORY ENFORCEMENT)

SENIOR EXAMINER RULE: The 4-Mark Comparison Formula 1HI0/11 · Section B Q3 Protocol

To score full **4/4 Marks** on Question 3: **1.** State 1 explicit similarity or difference in your opening sentence; **2.** Support with 1 precise historical fact from Period A; **3.** Support with 1 precise historical fact from Period B; **4.** Directly connect both with a comparative connective (*whereas, in contrast, similarly*). Do NOT write an essay—target 4 to 5 lines maximum!

DRILL 1 · SIMILARITY Ideas about Causes of Disease: Medieval England (c1250–c1500) [4] vs Renaissance (c1500–c1700)

Core Hook: Miasma / Four Humours *Model Stem: "One way in which ideas about causes of disease were similar was the continued belief in miasma; during the Black Death in 1348... similarly during the Great Plague in 1665..."*

DRILL 2 · DIFFERENCE Disease Prevention: Edward Jenner (1796) vs Louis Pasteur's [4] Germ Theory Vaccines (1880s)

Core Hook: Empirical Observation vs Laboratory Science *Model Stem: "One difference was that Jenner developed vaccination through empirical observation of cowpox without knowing germs caused disease, whereas Pasteur..."*

DRILL 3 · DIFFERENCE Hospital Care: Thirteenth-Century Monastic Hospitals vs [4] Nineteenth-Century Nightingale Hospitals

Core Hook: Spiritual Care vs Clinical Cure & Sanitation *Model Stem: "One key difference was that 13th-century hospitals run by monks focused on spiritual care and rest for travelers, whereas 19th-century hospitals..."*

SECTION B: MEDICINE IN BRITAIN, C1250–PRESENT

Paired Choice Timed Live Simulation · Question 5 OR Question 6 (16+4 Marks · Target: 25 Mins)

TIMED MOCK · QUESTION 5 OR 6

EXAM INSTRUCTION: ANSWER EITHER QUESTION 5 OR QUESTION 6. DO NOT ANSWER BOTH. TOTAL: 20 MARKS

☐ OPTION A: QUESTION 5 (c1250–c1700)

EDEXCEL JUNE 2019

'Attempts to prevent illness were completely ineffective between c1250 and c1700.' How far do you agree? Explain your answer.

Stimulus: • The Black Death (1348–49) • The Great Plague in London (1665) *(Plus own knowledge)*

☐ OPTION B: QUESTION 6 (c1900–present)

★ HIGH-YIELD FORECAST

'Understanding of the causes of disease advanced more rapidly in the twentieth century than in any other period.' How far do you agree?

Stimulus: • Discovery of DNA (1953) • Research into lifestyle (Doll & Hill, 1950) *(Plus own knowledge)*

1. CRITERIA JUDGEMENT State your explicit measurement criteria in your opening sentence (e.g. rate of scientific discovery vs real clinical impact).	2. BALANCED ARGUMENT Devote 1 paragraph to agreeing with the named statement, and 2 paragraphs exploring alternative factors or periods.	3. LEVEL 4 CLINCHER Your conclusion must resolve the tension: why was one factor ultimately more decisive than the others?
--	--	--

INTRODUCTION & PARAGRAPH 1: DIRECT ENGAGEMENT WITH STATEMENT / NAMED FACTOR

PARAGRAPH 2: COUNTER-ARGUMENT / SECOND STIMULUS FACTOR

PARAGRAPH 3: INDEPENDENT CONTEXT FACTOR (OWN KNOWLEDGE BEYOND STIMULUS POINTS)

CONCLUSION: JUSTIFIED CRITERIA-LED FINAL JUDGEMENT

SECTION A: THE HISTORIC ENVIRONMENT

The British Sector of the Western Front, 1914–18 · Questions 1(a), 1(b) (Features) & Question 2(b) (Follow-Up)

1 (a) EDEXCEL JUNE 2018 Describe one feature of the work of Casualty Clearing Stations (CCS) on the Western Front. [2]

KEY KNOWLEDGE (AO1)

Located 7–12 miles behind frontline near railway lines; staffed by RAMC doctors and QAIMNS nurses; performed triage (walking wounded, urgent surgery, beyond help); did life-saving amputations.

F-D EXAM FORMULA

Feature: State clear historical role in sentence 1 → **Detail:** Support with 1 precise fact, statistic, or procedure in sentence 2.

1 (b) EDEXCEL JUNE 2019 Describe one feature of the use of the Thomas splint on the Western Front. [2]

KEY KNOWLEDGE (AO1)

Designed by Hugh Owen Thomas; applied at Regimental Aid Posts; rigid steel frame pulled leg straight to prevent bone ends grinding and cutting femoral artery; reduced fracture mortality from 80% to 20%.

F-D EXAM FORMULA

Feature: State medical function in sentence 1 → **Detail:** Support with mortality drop (80% to 20%) or mechanical traction detail in sentence 2.

2 (b) EDEXCEL JUNE 2018 Study Source B (on Page 19). How could you follow up Source B to find out more about the severe conditions faced by medical staff at Base Hospitals? Complete the table below. [4]

Detail in Source B that I would follow up:	
Question I would ask:	
What type of source I would use:	
How this source would help me answer my question:	

Enquiry Verification Checklist: [] Direct quote from Source B selected; [] Question directly links to quote; [] Realistic historical source named (e.g. Base Hospital War Diary or RAMC inspection log); [] Clear purpose explained.

The British Sector of the Western Front, 1914–18 · Question 2(a) Source Utility Enquiry (Sources A & B)

SOURCE A	SOMME CCS	SOURCE B	BASE HOSPITAL
<i>"The casualties have started to arrive... The volume of wounded men is overwhelming; every bed and stretcher is occupied, and men are fortunate just to find a spot on the bare ground. Many will pass away before surgery is possible. We have admitted over 1,500 casualties in twenty-four hours, and the stream has not stopped."</i> Provenance: From the personal wartime diary of the Reverend Arthur Davies, an army chaplain stationed at a Casualty Clearing Station during the Battle of the Somme, July 1916.		<i>"Our morning duties begin at 3.30 am! The freezing temperatures are unbearable. Icicles hang thick over ward windows. Even kettles, rubber hot water bottles, and sponges have frozen solid. The pipes freeze entirely, meaning we must strictly ration the water provided to the wounded."</i> Provenance: From a private letter written by Edith Smith, a VAD (Voluntary Aid Detachment) nurse serving at a British Base Hospital near Boulogne, December 1917.	

PROVENANCE CLUES (AUTHOR, AUDIENCE, MOTIVE) — CONSIDER BEFORE WRITING:

- **Source A (Chaplain Davies):** Eyewitness chaplain at a CCS during the peak of the Somme (July 1916). *Motive:* Private diary recording immediate emotional and physical strain of 1,500 casualties arriving; highly reliable for firsthand volume, though written under immense pressure without official statistics.
- **Source B (Edith Smith):** Frontline VAD nurse writing a personal letter home from a Base Hospital in winter 1917. *Motive:* Candid description of extreme winter freezing and water rationing; highly useful for everyday environmental hardships, though localized to one coastal hospital.

C — CONTENT & QUOTE	O — OWN KNOWLEDGE	P — PROVENANCE (NOP)
Identify specific details (1,500 casualties, bare ground, frozen pipes, water rationing) and explain what they reveal about medical strain.	Corroborate with precise context: 57,000 casualties on Day 1 of Somme; CCS capacity was ~1,000; winter 1917 was coldest in 40 years.	Evaluate Nature (diary vs letter), Origin (eyewitness chaplain vs nurse), and Purpose. Weigh typicality vs limitations.

- **Content Starter:** *Source A is useful for an enquiry into casualty treatment because it shows that...*
- **Context Starter:** *From my own historical knowledge, this is accurate because during the 1916 Somme offensive...*
- **Provenance Starter:** *The utility of Source A is affected by its provenance because as an eyewitness chaplain's diary...*

[illegible]

The British Sector of the Western Front, 1914–18 · Question 2(a) Student Writing Response Lines

- **Content Starter:** *Source B is also useful because it highlights the severe environmental difficulties...*
- **Context Starter:** *This is corroborated by historical evidence that Base Hospitals in winter 1917 faced...*
- **Provenance & Synthesis:** *As a personal letter home from a VAD nurse, the provenance... Overall, Source [A/B] is more useful because...*

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1 (a)★ HIGH-YIELD FORECAST

Describe one feature of mobile X-ray units used in the British sector of the Western Front.

[2]

KEY KNOWLEDGE (AO1) 6 mobile X-ray vans deployed by RAMC in British sector; located at CCS; transported X-ray tubes in padded vans; used to locate shrapnel and bullets inside flesh before surgery; tubes overheated quickly.	F-D EXAM FORMULA Feature: State diagnostic role in sentence 1 → Detail: Add specific technological detail (van transport, overheating tubes, or CCS location) in sentence 2.
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1 (b)EDEXCEL JUNE 2022

Describe one feature of trench fever on the Western Front.

[2]

KEY KNOWLEDGE (AO1) Caused by micro-organisms living in body lice feces; produced severe headaches, high fever, and disabling bone pain in shins; affected ~15% of men; led to bathhouses and steam delousing machines.	F-D EXAM FORMULA Feature: State lice cause or flu-like symptoms in sentence 1 → Detail: Add delousing stations or 15% casualty impact in sentence 2.
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2 (b)★ HIGH-YIELD FORECAST

Study this extract from an RAMC officer at Ypres (1915): *"The chlorine gas casualties stumbled in clutching their throats. We washed their blinded eyes with bicarbonate of soda solutions, but many died from asphyxiation."* [4]

How could you follow up this extract to find out more about treatments for gas attacks on the Western Front? Complete the table below.

Detail in extract that I would follow up:	
Question I would ask:	
What type of source I would use:	
How this source would help me answer my question:	

Enquiry Verification Checklist: [] Direct quote from gas extract selected; [] Focused clinical enquiry question; [] Official RAMC Casualty Admission Book or War Office Bulletin named; [] Precise evaluative purpose.

The British Sector of the Western Front, 1914–18 · Question 2(a) Source Utility Enquiry (Sources C & D)

SOURCE C	FIELD AMBULANCE LOG
	<i>"The mud in the Ypres Salient makes stretcher bearing almost impossible. It takes four or six bearers to carry one stretcher through waist-deep slime. The wooden duckboards have been blasted away by German artillery. Yesterday it took our squad four hours to move two casualties just one mile back to the dressing station."</i> Provenance: From the official war diary of a British RAMC Field Ambulance unit operating during the Third Battle of Ypres (Passchendaele), August 1917.

SOURCE D	AMBULANCE TRAIN LOG
<i>"The ambulance train arrived at midnight carrying 400 lying cases directly from the clearing station. The train is fitted with sprung cots and electric lighting, but the stench of gangrene and damp uniforms is overpowering. Medical orderlies worked continuously dressing wounds and administering hot tea."</i> Provenance: From the wartime journal of Sister Kate Luard, a nursing sister serving aboard British RAMC Ambulance Trains between the Somme and Base Hospitals at the French coast, 1916.	

- **Source C (RAMC Field Ambulance Diary):** Official frontline operational log written by stretcher-bearer commanders at Passchendaele (1917). *Motive:* Recording military logistics and delay factors; highly factual regarding physical mud barriers, though focused strictly on frontline sector transport.
- **Source D (Sister Kate Luard Journal):** Experienced nursing sister serving on specialized ambulance trains. *Motive:* Eyewitness clinical observation of intermediate evacuation between CCS and Base Hospitals; highly valuable for understanding rail medical care, though limited to train-borne patients.

Extract specific evidence (waist-deep slime, 4 hours for 1 mile, 400 cases on sprung cots, gangrene stench) and analyse logistical hurdles.

Integrate evacuation chain: RAP → Dressing Station → CCS → Ambulance Train → Base Hospital. Mention motor ambulances and canal barges.

Contrast operational field log (Source C) with clinical nursing journal (Source D). Weigh author authority and scope of evidence.

[4 MARKS]

- **Content Starter:** *Source C is useful for an enquiry into frontline transport because it shows that...*
- **Context Starter:** *From my own knowledge, this reflects the extreme conditions during Third Ypres (Passchendaele)...*
- **Provenance Starter:** *The utility of Source C is influenced by its nature as an official RAMC Field Ambulance War Diary...*

The British Sector of the Western Front, 1914–18 · Question 2(a) Student Writing Response Lines

- **Content Starter:** *Source D is also useful because it details the next stage of the evacuation chain...*
- **Context Starter:** *Specifically, my knowledge confirms that specialized ambulance trains were vital because...*
- **Provenance & Synthesis:** *Sister Luard's firsthand clinical perspective makes this source... Overall, both sources are mutually useful because...*

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SECTION A: THE HISTORIC ENVIRONMENT

The British Sector of the Western Front, 1914–18 · Questions 1(a), 1(b) (Features) & Question 2(b) (Follow-Up)

1 (a) ★ HIGH-YIELD FORECAST Describe one feature of the underground hospital at Arras (Thompson’s Cave). [2]

KEY KNOWLEDGE (AO1) Constructed in chalk quarries by New Zealand and British miners (1916–17); contained 700 beds, running piped water, electric lighting, and fully sterile surgical theatres; completely protected staff and patients from German artillery fire.	F-D EXAM FORMULA Feature: State underground location & capacity in sentence 1 → Detail: Add technological infrastructure (electric dynamos, piped water, or shell protection) in sentence 2.
---	--

1 (b) EDEXCEL JUNE 2023 Describe one feature of the Carrel-Dakin method for treating wound infections on the Western Front. [2]

KEY KNOWLEDGE (AO1) Devised by Alexis Carrel and Henry Dakin (1915); sterilised sodium hypochlorite chemical solution flushed deep into wound through perforated rubber tubes; prevented gas gangrene and sepsis; solution had to be freshly prepared every 6 hours.	F-D EXAM FORMULA Feature: State chemical antiseptic action in sentence 1 → Detail: Describe continuous tube irrigation or fresh preparation requirement in sentence 2.
--	--

2 (b) ★ HIGH-YIELD FORECAST Study Source F (on Page 25). How could you follow up Source F to find out more about how surgeons dealt with wound infections and gangrene during the Battle of Arras? Complete the table below. [4]

Detail in Source F that I would follow up:	
Question I would ask:	
What type of source I would use:	
How this source would help me answer my question:	

Enquiry Verification Checklist: [] Quote on gangrene or surgical fatigue selected; [] Specific clinical question; [] Official RAMC Surgical Casebook or Medical Research Committee Report named; [] Clear purpose explained.

The British Sector of the Western Front, 1914–18 · Question 2(a) Source Utility Enquiry (Sources E & F)

SOURCE E	RAMC REPORT (ARRAS)
	<i>"The underground hospital in the chalk quarries of Thompson's Cave accommodates up to 700 stretcher cases. It is fitted with electric power dynamos providing light, running water, and fully equipped surgical theatres. This extensive network of chalk caves protects both medical officers and patients completely from heavy artillery bombardments, allowing continuous operations."</i> Provenance: From an official medical inspection report by Major C. S. Myers, an RAMC consulting physician inspecting the underground tunnels at Arras, April 1917.

SOURCE F **FRONTLINE MEMOIR**

"The wounded men brought down into the chalk tunnels were coated in lime dust, gashed by shrapnel, and suffering terribly from gangrene. In the underground wards, surgical teams worked without pausing day or night under glaring electric bulbs while the earth overhead vibrated from artillery fire. The damp air and smell of disinfectants was suffocating."

Provenance: From *The Forbidden Zone*, a published memoir by Staff Nurse Mary Borden, who operated an advanced surgical clearing unit near the Arras front in 1917.

- **Source E (Major Myers Report):** High-ranking RAMC officer writing an official inspection report on the Arras tunnel hospital in April 1917. *Motive:* Evaluating infrastructure and equipment for military commanders; highly reliable for capacity (700 beds), power dynamos, and physical safety, though focused on institutional success rather than clinical suffering.
- **Source F (Nurse Borden Memoir):** Frontline surgical nurse writing an eyewitness literary memoir. *Motive:* Communicating the visceral sensory reality of treating gas gangrene and shrapnel wounds underground; deeply insightful for physical working conditions, though written with dramatic retrospection.

Identify specific details (700 beds, electric dynamos, underground safety, chalk dust, vibrating earth, suffocating damp) and explain medical impact.

Corroborate with historical context: New Zealand tunneling company created 800m of tunnels; Arras April 1917; Carrel-Dakin method used for gangrene.

Contrast official RAMC administrative report (Source E) with personal frontline surgical memoir (Source F). Synthesize mutual utility.

[4 MARKS]

- **Content Starter:** *Source E is useful for an enquiry into medical facilities at Arras because it demonstrates...*
- **Context Starter:** *From my historical knowledge, this accurately reflects the construction of Thompson's Cave where...*
- **Provenance Starter:** *The utility of Source E is reinforced by Major Myers' position as an official RAMC inspector because...*

The British Sector of the Western Front, 1914–18 · Question 2(a) Student Writing Response Lines

- **Content Starter:** *Source F is also useful because it reveals the grim physical conditions inside the underground wards...*
- **Context Starter:** *This is supported by evidence that frontline surgeons battled severe gas gangrene and sepsis using...*
- **Provenance & Synthesis:** *While Mary Borden writes from personal nursing experience, both sources combine effectively because...*

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SECTION A: THE HISTORIC ENVIRONMENT

The British Sector of the Western Front, 1914–18 · Questions 1(a), 1(b) (Features) & Question 2(b) (Follow-Up)

1 (a) ★ HIGH-YIELD FORECAST Describe one feature of blood transfusion and storage used at the Battle of Cambrai (1917). [2]

KEY KNOWLEDGE (AO1) Captain Oswald Robertson established the first portable blood bank; universal Group O blood mixed with sodium citrate to prevent clotting and dextrose for energy; 22 units packed in ice and sawdust inside ammunition boxes.	F-D EXAM FORMULA Feature: State sodium citrate preservation or portable bank in sentence 1 → Detail: Add iced ammunition boxes or Cambrai 1917 trial in sentence 2.
--	--

1 (b) EDEXCEL JUNE 2021 Describe one feature of trench foot in the British sector of the Western Front. [2]

KEY KNOWLEDGE (AO1) Caused by prolonged standing in waterlogged, freezing mud causing restricted blood circulation; led to gangrene and amputation; prevented by rubbing whale oil on feet, changing into dry socks twice daily, and the 'buddy system' of inspection.	F-D EXAM FORMULA Feature: State waterlogged mud cause or gangrene danger in sentence 1 → Detail: Add whale oil or dry socks prevention in sentence 2.
--	--

2 (b) ★ HIGH-YIELD FORECAST Study Source G (on Page 28). How could you follow up Source G to find out more about the success of blood transfusions in treating wounded soldiers during the Battle of Cambrai? Complete the table below. [4]

Detail in Source G that I would follow up:	
Question I would ask:	
What type of source I would use:	
How this source would help me answer my question:	

Enquiry Verification Checklist: [] Direct quote on 11 transfused men selected; [] Focused clinical outcome question; [] Official Casualty Clearing Station No. 34 Admission Register or RAMC Surgical Journal named; [] Precise evaluative purpose.

The British Sector of the Western Front, 1914–18 · Question 2(a) Source Utility Enquiry (Sources G & H)

SOURCE G **BMJ REPORT (1918)**

"During the Cambrai offensive in November 1917, we established an advanced depot of preserved blood. Twenty-two units of universal donor Group O blood were collected in glass bottles containing sodium citrate and dextrose, kept packed in sawdust and ice within ammunition boxes. Eleven men suffering from extreme hemorrhage and surgical shock were transfused; nine recovered completely."

Provenance: From a scientific paper by Captain Oswald Robertson (US Army Medical Officer attached to British CCS No. 34), published in the *British Medical Journal*, June 1918.

SOURCE II **ORDERLY MEMOIR**

"Before preserved blood arrived, men bleeding heavily from severed femoral arteries or shattered limbs slipped rapidly into shock and death. When the medical officers began administering Robertson's bottled blood, the transformation was astonishing—pale, dying boys literally regained colour in their lips and warmth in their skin within twenty minutes."

Provenance: From the post-war memoir of Private Harold Devenish, an RAMC orderly serving at a Casualty Clearing Station near Cambrai in late 1917.

- **Source G (Captain Robertson BMJ):** Pioneering medical officer publishing scientific data in Britain's leading medical journal (1918). *Motive:* Demonstrating clinical efficacy of citrated blood to the medical community; provides precise scientific statistics (22 units, 11 transfusions, 9 recoveries), though written to promote his technique.
- **Source H (RAMC Orderly Devenish):** Frontline RAMC orderly observing immediate patient reactions in the wards. *Motive:* Recording personal recollections of medical miracles; offers valuable eyewitness evidence of physiological recovery from shock, though written years later without clinical records.

Extract specific facts (22 units preserved blood, sodium citrate, iced ammunition boxes, 9 of 11 recovered, regained colour in 20 mins) and evaluate treatment for shock.

Connect to Landsteiner (blood groups 1901), Hustin (sodium citrate 1914), and Rous & Turner (glucose/dextrose 1916). Explain why shock was lethal before transfusion.

Contrast scientific medical journal report (Source G) with nursing orderly's personal memoir (Source H). Weigh clinical data against visual observation.

[4 MARKS]

- **Content Starter:** *Source G is useful for an enquiry into treatments for blood loss because it demonstrates that...*
- **Context Starter:** *From my historical knowledge, Captain Robertson's work at Cambrai in 1917 was groundbreaking because...*
- **Provenance Starter:** *The utility of Source G is elevated by its provenance as a British Medical Journal report because...*

The British Sector of the Western Front, 1914–18 · Question 2(a) Student Writing Response Lines

PARAGRAPH 2: ANALYSIS OF SOURCE H & COMPARATIVE JUDGEMENT (CONTENT + KNOWLEDGE + PROVENANCE)

- **Content Starter:** *Source H is also useful because it confirms the dramatic real-time clinical effect on soldiers in shock...*
- **Context Starter:** *This is supported by evidence that rapid blood replacement raised blood pressure and prevented...*
- **Provenance & Synthesis:** *Private Devenish's eyewitness observations corroborate the BMJ data... Overall, both sources provide...*

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ARCHIVAL PROVENANCE PROTOCOL: EVALUATING HISTORICAL EVIDENCE Edexcel
Specification Option 11

In Question 2(a) [8 Marks] and Question 2(b) [4 Marks], examiners require candidates to evaluate sources in historical context using **Nature, Origin, and Purpose (NOP)**. Never dismiss a source as "biased" or "useless". Instead, explain how its specific archival context dictates what it can and cannot reveal about medical treatments, casualty logistics, and conditions.

SOURCE TYPOLOGY & EXAMPLE	ARCHIVAL PURPOSE (MOTIVE)	CORE HISTORICAL STRENGTHS (HIGH UTILITY)	EVALUATIVE LIMITATIONS & BLIND SPOTS
RAMC Unit War Diaries e.g. Field Ambulance & CCS Logs (TNA: WO 95)	Official daily operational record written by commanding medical officers for War Office command.	- Extremely reliable for precise casualty counts, arrival times, and logistical bottlenecks. - Records official movements, equipment shortages, and structural reorganizations.	- Downplays panic, panic-induced errors, or administrative failure to protect military reputation. - Omits visceral individual soldier suffering and personal emotional reactions.
Hospital Admission Books e.g. Medical Records (TNA: MH 106)	Statistical and administrative ledger tracking individual admissions, wound types, treatments, and discharges.	- Unmatched empirical accuracy for tracking specific wounds (gunshot, shrapnel, gas, trench foot). - Verifies survival rates and average length of stay across different hospitals.	- Purely administrative; provides zero narrative explanation of clinical methods or surgical challenges. - Incomplete during chaotic mass offensives (e.g. Somme Day 1).
Personal Wartime Letters & Diaries e.g. VAD Nurses, Chaplains & Stretcher-Bearers	Private, uninhibited personal reflection or communication with family back home in Britain.	- Invaluable eyewitness evidence of ward atmosphere, physical exhaustion, and grim sensory details. - Reveals true feelings of staff facing horrific mutilations and freezing conditions.	- Subject to military postal censorship (letters often avoided depressing facts to spare relatives). - Highly localized to one specific aid post or ward; lacks broad statistical perspective.
Official Trench Photographs e.g. Imperial War Museums (IWM Q Series)	Official visual record taken by British official war photographers (e.g. John Warwick Brooke) for public release.	- Verifiable primary visual evidence of medical machinery, uniform condition, mud terrain, and stretcher gear. - Depicts layout of advanced dressing stations and ambulance trains.	- Heavily staged and curated for propaganda; never depicted dying men, amputations, or catastrophic failure. - Camera shutter speeds could not capture real-time battlefield chaos.
Professional Medical Journals e.g. The Lancet & British Medical Journal (1914–18)	Scientific exchange among senior surgeons and bacteriologists to evaluate clinical techniques and trial results.	- Rigorous scientific descriptions of innovations (Carrel-Dakin method, Thomas splint, stored blood). - Provides precise clinical trial data and comparative mortality statistics.	- Authors often had professional egos and exaggerated the success of their proprietary techniques. - Reflects cutting-edge hospital practice rather than standard frontline trench realities.
Post-War Memoirs & Oral Testimonies e.g. Published Books (1920s–1980s)	Retrospective recollection written years or decades after the war for historical remembrance or commercial publication.	- Free from wartime censorship; candid commentary on military incompetence and medical disasters. - Reflective overview of the entire conflict and psychological impact.	- Vulnerable to memory fade, hindsight bias, and dramatic exaggeration for reader entertainment. - Often conflates different battles or repeats popular post-war myths.

HOW TO APPLY THIS MATRIX IN QUESTION 2(A) SOURCE UTILITY [8 MARKS]:

When answering Q2(a), link the **nature of the source** directly to the specific enquiry. For example: *"Because Source A is an official RAMC Field Ambulance war diary (WO 95), its operational nature makes it exceptionally useful for establishing the exact delays caused by the Passchendaele mud; however, its official purpose means it omits the emotional despair recorded in private nursing memoirs like Source B."*

1. THE "DEALT VS FELT" EXAM TRAP (SECTION A) Common Level 1/2 Pitfall

The Examiner Warning: Every year, thousands of students lose half the available marks in Section A because they write about how soldiers *felt* in the trenches (mud, fear, rats, lice, miserable food) rather than how the RAMC *dealt* with injuries, treatments, and medical logistics.

The Golden Rule: The historic environment specification is titled "*Injuries, treatment and the trenches*." Always keep your sentences anchored to medical interventions: triage systems, Thomas splints, mobile X-rays, debridement, Carrel-Dakin antiseptic irrigation, citrated blood transfusions, and specialized evacuation transport.

2. THE 3-ASPECT RULE FOR QUESTION 4 [12 MARKS] Section B Analytical Architecture

The Examiner Warning: Edexcel mark schemes explicitly state that candidate responses relying *only* on the two printed stimulus points cannot achieve Level 4 (maximum 8/12 marks). You **MUST** introduce relevant own knowledge.

The Golden Rule: Structure every 12-mark answer into exactly **3 P-E-E paragraphs (3 distinct aspects)**:

- **Paragraph 1 (6 lines):** Stimulus Point 1 + deep factual explanation + analytical causal link.
- **Paragraph 2 (6 lines):** Stimulus Point 2 + deep factual explanation + analytical causal link.
- **Paragraph 3 (6 lines):** Independent Own Knowledge Factor (e.g. enfranchisement, Royal Society, corn steep liquor).

3. QUESTION 2(a) [8 MARKS]: COMPARATIVE SYNTHESIS & MUTUAL UTILITY Section A Source Enquiry

The Examiner Warning: Candidates who evaluate Source A and Source B in complete isolation without ever comparing them struggle to reach the top of Level 3.

The Golden Rule: Always follow the **C-O-P Formula** (Content, Own Knowledge, Provenance) for both sources, and finish Paragraph 2 with a **synthesis judgement**: explain how Source A and Source B are *mutually supportive* or how one source provides the essential context missing from the other (e.g. an official war diary confirms the operational scale, while a nurse's diary reveals the human ward reality).

4. QUESTION 2(b) [4 MARKS]: THE PERFECT 4/4 FOLLOW-UP ALGORITHM Section A Follow-Up Enquiry

The 4-Step Algorithm:

1. Detail in Source:	Quote a short, precise phrase from the text (e.g. " <i>pipes freeze entirely... strictly ration the water</i> ").
2. Question to Ask:	Must directly investigate the quote (e.g. " <i>How did freezing winter temperatures affect patient mortality at Base Hospitals?</i> ").
3. Type of Source:	Name a realistic historical document: National Archives RAMC Base Hospital War Diary or Casualty Admission Book . NEVER write "the internet" or "a history textbook".
4. How it Helps:	Explain how the source answers the specific question (e.g. " <i>It would provide official mortality statistics and medical logs recording hypothermia cases during winter 1917</i> ").

SECTION B: THEMATIC STUDY (1)**UNIT 1: C1250–C1500: MEDICINE IN MEDIEVAL ENGLAND****IDEAS ABOUT THE CAUSE OF DISEASE AND ILLNESS:**

- ☐ Supernatural and religious explanations of the cause of disease.
- ☐ Rational explanations: Theory of Four Humours and miasma theory.
- ☐ The continuing medical influence of Galen in England.

APPROACHES TO PREVENTION AND TREATMENT:

- ☐ Religious actions, bloodletting, purging, purifying the air.
- ☐ Roles of physician, apothecary, barber surgeon, hospitals & home remedies.

CASE STUDY:

- ☐ **Dealing with the Black Death, 1348–49:** treatments and prevention.

UNIT 2: C1500–C1700: THE MEDICAL RENAISSANCE**IDEAS ABOUT THE CAUSE OF DISEASE AND ILLNESS:**

- ☐ Continuity and change; scientific observation (Thomas Sydenham).
- ☐ Impact of printing press and the work of the Royal Society.

APPROACHES TO PREVENTION AND TREATMENT:

- ☐ Continuity and change in care; hospitals and community herbalists.
- ☐ Anatomical training and the influence of Vesalius.

CASE STUDIES:

- ☐ **William Harvey:** circulation of the blood.
- ☐ **Great Plague (1665):** treatments and attempts to prevent spread.

SECTION B: THEMATIC STUDY (2)**UNIT 3: C1700–C1900: 18TH & 19TH CENTURY BRITAIN****IDEAS ABOUT THE CAUSE OF DISEASE AND ILLNESS:**

- ☐ Pasteur's Germ Theory (1861) and its influence; Robert Koch's bacteriology.

APPROACHES TO PREVENTION AND TREATMENT:

- ☐ Surgical improvements: Simpson (chloroform), Lister (carbolic), asepsis.
- ☐ Hospital care and nursing reform: Florence Nightingale.
- ☐ Public Health Acts of 1848 and 1875; municipal sanitation.

CASE STUDIES:

- ☐ **Edward Jenner:** smallpox vaccination.
- ☐ **John Snow:** Broad Street cholera pump (1854).

UNIT 4: C1900–PRESENT: MODERN MEDICINE**IDEAS ABOUT THE CAUSE OF DISEASE AND ILLNESS:**

- ☐ Genetics and DNA (Watson, Crick, Franklin); Human Genome Project.
- ☐ Lifestyle factors: smoking (Doll & Hill), diet, alcohol.
- ☐ Advanced diagnostic physics: X-rays, CT scans, MRI, blood testing.

APPROACHES TO PREVENTION AND TREATMENT:

- ☐ Magic bullets (Salvarsan 606, Prontosil) & antibiotics (Penicillin).
- ☐ High-tech surgery: transplants, keyhole surgery, prosthetics.
- ☐ State healthcare: National Health Service (1948) & vaccination campaigns.
- ☐ Government lifestyle legislation: Clean Air Acts, smoking bans.

SECTION A: HISTORIC ENVIRONMENT**THE BRITISH SECTOR OF THE WESTERN FRONT, 1914–18****CONTEXT & THEATRE OF WAR:**

- ☐ Ypres salient, the Somme, Arras chalk tunnels, Cambrai.
- ☐ Trench system: front, support, communication trenches, terrain & mud.
- ☐ Transport problems: stretcher-bearers, motor ambulances, trains, barges.

INJURIES, ILLNESSES & TREATMENTS:

- ☐ Illnesses: trench foot, trench fever (body lice), shell shock (NYDN).
- ☐ Weapons effects: shrapnel wounds, bullet wounds, gas (chlorine, phosgene, mustard).
- ☐ Infection: gas gangrene, tetanus, debridement, Carrel-Dakin method.
- ☐ Fracture treatment: Hugh Owen Thomas splint (mortality dropped 80% to 20%).
- ☐ Medical technology: mobile X-ray vans (locating shrapnel/bullets).
- ☐ Blood transfusions: sodium citrate (Hustin), glucose storage (Rous/Turner), Robertson's Cambrai blood bank (1917).

CHAIN OF EVACUATION:

- ☐ Regimental Aid Posts (RAP)
- ☐ Advanced & Main Dressing Stations (Field Ambulance)
- ☐ Casualty Clearing Stations (CCS) — triage & emergency surgery
- ☐ Base Hospitals — specialist surgery & convalescence
- ☐ Underground hospital at Arras (Thompson's Cave: 700 beds)
- ☐ Medical personnel: RAMC, FANY, QAIMNS, VAD nurses